

Module 1 Reflection | [Deb Gorombej] | [06/21/2026]

Aim for 80–130 words per prompt (250–400 words total). Use the sentence starters if you are stuck.

In the final 5 minutes, re-read and underline one sentence you want to remember or discuss. Do not edit for style. This is a thinking document — clarity of idea matters; polish does not

What assumption about AI did this module confirm, challenge, or complicate? Describe the assumption and what changed. (Sentence starter: 'Before this module, I assumed AI... After reading/exploring, I now understand...')

Before this module, I had a more superficial understanding of how AI systems were developed and about the existence of the terms 'foundational model' and how these 'pre-trained' models are adapted to for efficiency and to reduce 'recreating the wheel.' This puts what I previous knew about AI in greater perspective. After reading, I discovered that I need more time to process new terms that built upon what I previously knew. I had to look up some new terms such as BERT and CLIP. After the readings, I am now even more cautious of the trajectory of AI due to emergence, lack of control, 'brakes' and fact checking. Now I see the AI world is more challenging and I have begun to think of ways I may be able to parse the information down or find some other sources to provide faculty and students when I am teaching about AI in healthcare so they are not scared away and drop my upcoming course. I will be sure to use a workload estimator when making assignments for readings, projects and learning new software [ex. <https://cat.wfu.edu/resources/workload2/>].

What one aspect of how LLMs work technically surprised you most — and why does it matter for your professional context? (Sentence starter: 'I was surprised to learn that LLMs... This matters for my work because...')

I was surprised to learn how much academic involvement is going to training and utilization of AI systems. This matters for my work, because when myself or colleagues are using platforms need to still verify but also understand that data is being pulled from a variety of sources that can affect clinical decision making. An example, that was further down in the readings, but not required shows clinical data sources. I would like to learn more about how are clinical AI platforms constructed and how are they emerging / homogenizing, etc. I know clinicals providers are pressed for time (and often held to quotes and patient visit time limits) and an AI 'shortcut may not be prudent as we are held to guideline standards and cannot say 'AI told me to do xyz.' Further questions include: who / what checks the architecture in a model etc.?

I was also surprised to be reading the Bommasani et al. article and noticing that there were times that some paragraphs were not supported or cites (which affected my annotated table). This does however make sense as we are experiencing new frontiers in the world of AI real-time development. I was also intrigued and surprised about how I

previously thought about how AI terms fit in the larger AI space. I learned that a 2D model is insufficient for a 3D topic! I have to keep reinforcing these terms so I can have a better understanding as we going along in this course.

I was also surprised to see that it took me more than 12 hours to complete this module. I still do not have a great understanding of Github and the suggested concept map software. I am more used to D2L. I feel that I was underprepared for this course.

Name one specific recurring task in your role that you now think could be affected by current AI. Be concrete: name the task, identify the AI mechanism, and describe one realistic impact scenario — positive or cautionary. (Sentence starter: 'One task I perform regularly is [specific task]. Based on what I learned, [specific AI mechanism] could [specific impact]...')

One recurring task that occurs in my work with students is when I have a variety of roles. For some students, I am an:

1. Academic adviser
2. DNP project committee chair
3. Project second committee member
4. Project a third committee member
5. Clinical supervising faculty (CSF) with grading logs
6. CSF – Site visits with midterm check ins multiple different clinical sites across the U.S. (Some students have 2 or more sites during a semester).
7. Contact person for clinical preceptors
8. Course chair for one or more didactic or clinical courses
9. Grader for one or more didactic or clinical courses
10. Mentor (informal)
11. Requested informally for project input

For each of these roles, I need to identify the student's background, Clinical specialty or dual specialties, if they are full time or part time, experience in Nursing/healthcare, doctoral project interest, when they are starting clinicals, where they are performing clinicals, state they are located and time zone, when they are graduating with initial degree and or additional certifications and what clinical field they are most interested in for their specialty rotation, what clinical specialty they are most interested in for their first job, which certification exam they which to take after graduation. Taking student data from application information and course feedback and grades may also help identify when more student support is needed.

Based on what I learned, BERT may be able to help with this deeper understanding and student support. I wonder if this may already exist in the University.

Feedforward mechanisms can be also helpful.

This will streamline faculty workflow, organize information and speed efficiency. A caution is that I am not sure of the process to utilize student data in this format.